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Course: HLSC 2P21:

TOPIC: FINAL REFLECTIVE PORTFOLIO

Meta-Reflection

Looking back on my experience in HLSC 2P21, I can see how much my understanding of health equity and justice has grown. The three CTAs I chose were on art as a tool for advocacy, the legacy of residential schools and the creation of Medicare may seem different, but they are deeply connected by the idea of addressing inequities in healthcare. They all taught me how injustice operates at individual, community, and extensive levels, while showing the importance of advocacy, action, and change.

When I started reflecting on these CTAs, I realized how emotional they were. For example, in the CTA about art and health justice, I felt anger at first. The injustice represented in the story felt overwhelming, but as I learned about people using art to inspire action, that anger turned into hope. One reading (Duffin, 2018), helped me see how grassroots movements whether through art or activism can create lasting change. This made me think about how even small efforts, like raising awareness through creative means, can contribute to a bigger movement for justice.

In the CTA about residential schools, I felt a mix of sadness, guilt, and curiosity. Peters story about the trauma caused by colonization was deeply personal and eye-opening. It made me reflect on how systemic issues, like the ongoing lack of clean water in Indigenous communities, are not just historical problems but they persist till today. This connected directly to the reading Indigenous Knowledge is the Solution to Canada's Health Inequities (2024), which emphasized the importance of integrating traditional knowledge and listening to Indigenous voices. The CTA on Medicare took me in a different direction. Learning about how World War I exposed Canadas poor healthcare infrastructure surprised me. The reading Why Increasing Canada's Health Budget for Preventive and

Social Care Makes Sense (2024) helped me connect the history of Medicare to current challenges like long wait times and unequal access in rural areas. I felt both admiration for Medicare's creation and frustration that some of these issues still have not been resolved. Reflecting on this, I realized that progress often comes with ongoing challenges that require attention and reform.

What surprised me most was how connected these themes were. Initially, I thought of them as separate topics, but through reflection, I saw how they all centered on justice whether it was through creative expression, reconciliation, or equal healthcare. Early in the course, we discussed the social determinants of health, and these ideas came up again in every CTA. For instance, art as advocacy addresses the social determinants of health by raising awareness and making volumes for unheard voices. Similarly, Medicare and Indigenous health intersect with equity and access, showing that systemic change is important for addressing these inequalities (Raphael, 2016).

This course did not just influence my understanding of healthcare, it also connected to my other classes and how I engage with the world. In my health and behavior class, we talked about how systemic factors shape individual choices, which aligns with what we learned in HLSC 2P21 about how colonization and inequity shape communities. When I read the news now, I see these systemic barriers more clearly. The emotions I felt during these CTAs were anger, sadness, curiosity which pushed me to focus more deeply. At times, I felt overwhelmed, especially when learning about the trauma of residential schools. But I used those feelings as motivation to learn and reflect. I am proud of how much effort I put into these assignments because they challenged me to think critically and personally. If there is one thing I would improve, it is the depth of my analysis in the Medicare CTA. I rushed through parts of it and missed opportunities to explore its connections to preventive care and equity.

This course has had an impact on how I see myself and my future. Personally, it challenged my assumptions and made me think about what justice really means. If I were in a medical school interview, I would share how this course taught me to think beyond individual

patients and consider the systems that affect health outcomes. I would emphasize the importance of empathy, action, and understanding systemic barriers.

In conclusion, these three CTAs taught me that health equity is not just about policies or practices, but it is about people. Whether through art, history, or reconciliation, each lesson showed me that justice requires both systemic change and personal accountability. Moving forward, I want to carry these lessons into my personal and professional life, advocating for a healthcare system that serves everyone, not just those with privilege.

Discrimination and Health: How is racism a health threat in Canada?

Racism is one of the hidden dangers in Canada healthcare system. It affects people's health not just through individual acts of discrimination but also through how the system is built. For example, Black Canadians and Indigenous people often experience racism when they seek medical help. Sometimes their pain is not taken seriously, or they're treated with less care than others. This can lead to worse outcomes, like untreated illnesses or even death (Mahabir et al., 2021).

The social determinants of health factors like income, education, and housing are also shaped by racism. For example, many racialized groups face barriers to getting high-paying jobs, safe housing, or quality education. This keeps them in situations where their health is at risk. For instance, studies show that racial minorities often live in neighborhoods with fewer healthcare facilities and more environmental hazards like pollution (Smedley et al., 2003). This shows that racism is not just a personal bias but it's a systemic issue that impacts entire communities.

I was especially struck by the story of Yosif Al-Hasnawi, a brock student whose death could have been prevented if paramedics had taken his injuries seriously (Hamilton Spectator, 2022). It made me angry and sad to see how racism can lead to such tragic outcomes. Reading about this case and others made me think about how accountability in healthcare could save lives. For example, implicit bias training for healthcare workers is a step in the right direction, but it needs to go beyond just awareness. There should be policies that ensure everyone gets fair treatment, regardless of their race (Bondzi-Simpson, 2022). This learning made me reflect on the importance of speaking up and challenging the systems that allow racism to continue. Moving forward, I want to advocate for policies that address the root causes of health disparities and ensure everyone gets the care they need.

Colonization and Indigenous Health: How does colonization shape the health of First Nations, Métis, and Inuit peoples in Canada?

Colonization has caused deep and lasting harm to the health of Indigenous peoples in Canada. Residential schools, where children were taken from their families and forced to abandon their culture, caused trauma that still affects Indigenous communities today. This trauma has been passed down through generations, leading to higher rates of mental health issues, addiction, and suicide (Kirmayer et al., 2011).

Even now, colonization's effects are everywhere. Many Indigenous communities lack necessities like clean water and proper healthcare. Imagine not being able to drink from your own tap without worrying about getting sick, that's the reality for many Indigenous people. Systemic racism in healthcare also means that Indigenous patients are often treated poorly or ignored, which discourages them from seeking help when they need it most (Mahabir et al., 2021). These issues show that colonization isn't just in the past, but it's still happening today.

What stood out to me the most was the strength and resilience of Indigenous communities. Despite everything, they've kept their culture alive through traditional healing practices and knowledge. This was highlighted in Indigenous Knowledge is the

Solution to Canada's Health Inequities (2024), which showed how combining traditional medicine with modern healthcare can lead to better outcomes. It reminded me of Peters story in class, where he talked about the importance of listening to Indigenous voices and taking action instead of just feeling sorry. This learning made me realize that reconciliation is about more than saying sorry—it's about making real changes. This could mean funding for clean water systems, better healthcare access in remote areas, and education programs that teach the true history of colonization. These steps are necessary to create a fairer healthcare system for Indigenous peoples.

Pressing Issues and Solutions: What are the biggest challenges in Canada's healthcare system, and how can we make it more equitable and sustainable?

Canada's healthcare system has many strengths, like universal access, but it also has big challenges. One of the biggest problems is the wait times. People in rural areas, for example, might wait months to see a doctor or get a prescription. This delay can make health issues worse and increase costs in the long run (Armstrong & Armstrong, 2016).

Another issue is equity. Some groups, like asylum seekers and Indigenous peoples, face barriers to accessing care. This could be because of language differences, racism, or a lack of nearby services. For instance, an asylum seeker might struggle to understand how to book an appointment or navigate the system. These challenges show that even though Canada's healthcare is universal, it's not always equal.

One solution is investing in preventive care. This means focusing on keeping people healthy before they get sick, like through regular check-ups, mental health support, and education campaigns. The reading Why Increasing Canada's Health Budget for Preventive and Social Care Makes Sense (2024) explains how this could reduce long-term costs and improve health outcomes. Another solution is expanding telehealth, especially in rural

areas where access is limited. Telehealth allows people to talk to doctors without traveling far, which saves time and money.

Addressing the social determinants of health is also very Important. Improving housing, education, and job opportunities can help reduce health disparities. For example, better housing can prevent illnesses related to mold or overcrowding (Raphael, 2016). These solutions take time and effort, but they're necessary for a fair and sustainable healthcare system. This learning made me think differently about healthcare. It's not just about fixing problems; it's about preventing them. Moving forward, I want to support policies and practices that make healthcare more equitable and accessible for everyone.

Lesson Plan: Health Equity and Indigenous Health in Canada

Lesson Title: "Indigenous Health and the Path to Equity: Reconciliation through Healthcare"

Duration: 90 minutes

Learning Objectives: By the end of this lesson, students will:

- Understand the historical and ongoing impact of colonization on Indigenous health.
- Explore the concept of cultural safety and its importance in Indigenous healthcare.
- Analyze how health equity can be achieved by integrating traditional knowledge with modern healthcare practices.

Materials Needed

- Projector and screen for slides/video
- Handouts: Summary of key readings, including Indigenous Knowledge is the Solution to Canada's Health Inequities
- Short documentary or video about Indigenous health in Canada (The Water Crisis in Indigenous Communities)
- whiteboard for group activities

Lesson Structure

Introduction (10 minutes)

Welcome and introduction to the topic: "Why is healthcare in Indigenous communities different?"

Briefly introduce the concept of health equity and why it matters for marginalized groups, focusing on Indigenous populations.

Play a short video showing the health disparities in Indigenous communities (lack of clean water, higher disease rates). This video will help the students emotionally connect with the topic.

Lecture/Presentation (25 minutes)

- Historical context: Discuss how colonization and residential schools have impacted Indigenous health (Kirmayer et al., 2011).
- Explain the social determinants of health and how they particularly affect Indigenous communities (Raphael, 2016).
- Introduce cultural safety: what it means and how healthcare providers can ensure they are creating safe spaces for Indigenous patients (Macaulay et al., 2011).

- Examples of success: Present examples where traditional knowledge has been integrated with Western medicine, showing the positive health outcomes (e.g., the role of Indigenous healers in modern health systems).

Group Discussion(20 minutes)

Case Study: Present a real-life scenario where an Indigenous community implemented traditional health practices to address a local health crisis (the use of traditional healing alongside modern healthcare during health outbreaks).

Divide students into small groups. Each group discusses one key question:

- "How can healthcare systems include Indigenous knowledge in a respectful way?"
- "What is one example of how healthcare could be more culturally safe for Indigenous people?"
- "How can we address the ongoing effects of colonization in healthcare delivery?"

After 10 minutes of group discussion, each group should present their ideas.

Interactive Activity: Role-Playing (30 minutes)

Scenario: Students will role-play as healthcare providers working with an Indigenous community to create a culturally safe healthcare plan. The scenario will involve addressing a health issue (e.g., diabetes or mental health) while considering both Western and traditional approaches. The goal is for students to integrate their learning about cultural safety, health equity, and reconciliation. Each group will present their plan to the class, explaining how they would involve Indigenous perspectives, cultural practices, and modern healthcare methods.

Reflection and Wrap-Up (10 minutes)

Conclude with an open discussion: "What actions can healthcare professionals take to ensure that Indigenous voices are central to healthcare decision-making?"

Ask students to write down one thing they learned today and how it will affect their future career path.

Assessment

Group Presentations: Each group will present their healthcare plan, showing how they integrated both cultural safety and health equity.

Reflection Journal: At the end of the lesson, students will be given a journal prompt to reflect on what they learned and how they can apply it in real-life situations.

Suggested Readings and Resources

Martin, D. (2018, November 28). Indigenous knowledge is the solution to Canada's health inequities. The Conversation. https://theconversation.com/indigenous-knowledge-is-the-solution-to-canadas-health-inequities-106226

Greenwood, M. L., & Naomi, S. (2024). Social determinants of health and the future well-being of Aboriginal children in Canada. Paediatrics & Child Health, 17(7), 381. https://pmc.ncbi.nlm.nih.gov/articles/PMC3448539/

Kirmayer, L. J., Gone, J. P., & Moses, J. (2014). Rethinking Historical Trauma. Transcultural Psychiatry, 51(3), 299–319. https://doi.org/10.1177/1363461514536358

2-Page Explanation

Introduction

This lesson plan was inspired by the syllabus in HLSC 2P21, especially the lessons on health equity and Indigenous health. The goal of this lesson is to introduce students to the challenges that Indigenous communities face in accessing healthcare, and how those challenges are rooted in colonization. I wanted students to not only learn about these challenges but also to think critically about ways they can be part of the solution.

Throughout the course, we have discussed the importance of social determinants of health (Raphael, 2016) and the impact of colonization on healthcare. By linking these ideas

to the specific struggles of Indigenous communities, students can see the direct connection between historical injustices and current health disparities. The readings like Indigenous Knowledge is the Solution to Canada's Health Inequities (2024) and Rethinking Historical Trauma (Kirmayer et al., 2011) have guided my understanding of how to approach health equity in Indigenous contexts.

One of the key lessons in this course is how traditional knowledge can improve healthcare outcomes. We discussed how colonialism has removed Indigenous healing practices from the healthcare system, and I wanted this lesson plan to address how these practices should be integrated back into healthcare in a respectful way. By involving both traditional knowledge and modern medicine, we can create a system that is more comprehensive and culturally safe for Indigenous people.

The issues addressed in this lesson are part of a global conversation about health equity. Whether it's Indigenous people in Canada, Aboriginal communities in Australia, or marginalized populations in other parts of the world, the challenge of achieving equitable healthcare is universal. This lesson shows how the healthcare system must evolve to be more inclusive, not just for Indigenous people but for all marginalized communities.

This lesson aligns with the goals of HLSC 2P21 by encouraging students to think beyond the classroom and consider how they can contribute to creating a healthcare system that is more just and equitable for everyone. I hope that by integrating cultural safety and equity into their thinking, students will become more empathetic and aware healthcare professionals in the future.

APPENDIX:

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